





1. ‘ForEVer #1 For ViCtory’ — FEV1

WHAT YOU SEE

The red banner reading 'ForEVer #1 For ViCtory!'.

WHAT IT ENCODES

FEV1 = forced expiratory volume in 1 second. A reduced FEV1/FVC ratio (<0.70 post-bronchodilator) defines obstruction; FEV1 % predicted then grades severity as GOLD 1 (>=80%), 2 (50-79%), 3 (30-49%), 4 (<30%). FEV1 is the 'O' in the BODE prognostic index.

BOARD TRAP

Diagnosis needs the FEV1/FVC RATIO <0.7 (POST-bronchodilator), not FEV1 alone. FEV1 % grades severity but does NOT, by itself, select the inhaler regimen.

2. ‘Visit for Capacity’ — spirometry

WHAT YOU SEE

The 'Visit for Capacity' sign in the rink.

WHAT IT ENCODES

Spirometry is required to diagnose COPD and is the objective test of airflow. Expect a reduced FEV1/FVC with INCREASED TLC and RV (air trapping/hyperinflation) and a REDUCED DLCO in emphysema (alveolar destruction lowers diffusing capacity); DLCO is typically normal in pure chronic bronchitis.

BOARD TRAP

COPD can't be diagnosed on symptoms or chest X-ray alone — spirometry is mandatory. A reduced DLCO points to emphysema, distinguishing it from chronic bronchitis.

3. GOLD CUP — GOLD staging & groups

WHAT YOU SEE

The Stanley-Cup-style 'GOLD CUP' trophy with gold medals.

WHAT IT ENCODES

GOLD grades 1-4 by FEV1 % predicted describe airflow severity, while the A/B/E groups (built from symptom scores — mMRC/CAT — plus exacerbation history) guide INITIAL inhaler therapy: Group A = single bronchodilator, Group B = LABA+LAMA, Group E = LABA+LAMA (+ICS if eos >=300).

BOARD TRAP

Inhaler choice is driven by symptoms + exacerbations (ABE groups), NOT by the spirometric 1-4 grade alone.

4. Pink player — ‘pink puffer’ (emphysema)

WHAT YOU SEE

The player dressed in pink.

WHAT IT ENCODES

Pink puffer = emphysema: progressive dyspnea, pursed-lip breathing, barrel chest, thin/cachectic build, hyperinflation, low DLCO, and relatively maintained oxygenation (they 'puff' to keep PaO2 up). Pathology is alveolar wall destruction with loss of elastic recoil.

BOARD TRAP

Emphysema = the 'pink puffer' — don't swap it with the cyanotic 'blue bloater.' Pink puffers are typically thin with preserved oxygenation until late.

5. Blue goalie — ‘blue bloater’ (chronic bronchitis)

WHAT YOU SEE

The goalie in blue at the far left.

WHAT IT ENCODES

Blue bloater = chronic bronchitis, a CLINICAL diagnosis: productive cough on most days for >=3 months/year over 2 consecutive years. Mucus gland hyperplasia (increased Reid index) → airway obstruction, cyanosis, hypoxemia, secondary erythrocytosis, cor pulmonale, and fluid retention ('bloated').

BOARD TRAP

Chronic bronchitis is a CLINICAL (cough-duration) diagnosis, not a spirometric or pathologic one. Blue bloaters are hypoxemic/cyanotic and prone to cor pulmonale.

6. Wind instruments — air trapping

WHAT YOU SEE

The brass band carrying tubas/sousaphones.

WHAT IT ENCODES

Airflow limitation → prolonged expiration and air trapping → hyperinflation with a flattened diaphragm, increased AP (barrel) chest diameter, and increased retrosternal air on imaging. Dynamic hyperinflation worsens with exertion and drives exertional dyspnea.

7. Lettered jerseys — bronchodilators (LAMA/LABA)

WHAT YOU SEE

Red-jersey players marked with letters.

WHAT IT ENCODES

Long-acting bronchodilators are the maintenance backbone: LAMA (e.g., tiotropium, blocks M3) and/or LABA (e.g., salmeterol/formoterol). SABA (albuterol) +/- SAMA (ipratropium) are for acute RESCUE and exacerbations.

BOARD TRAP

Long-acting agents are MAINTENANCE — they are NOT for acute rescue (that's the SABA). Conversely, don't manage chronic symptoms with SABA alone.

8. Inhaled corticosteroid (ICS)

WHAT YOU SEE

An extra lettered red-jersey player added beside the LAMA/LABA line — the bolt-on third teammate IS the inhaled corticosteroid that gets ADDED onto bronchodilators, never sent in to play alone.

WHAT IT ENCODES

Add an inhaled corticosteroid (to LABA/LAMA) for frequent exacerbations or high blood eosinophils (>=300 cells/μL strongly favors it; <100 favors against). ICS is always combined, building toward triple therapy in Group E exacerbators.

BOARD TRAP

ICS raises PNEUMONIA risk and is NOT first-line for everyone — reserve it for eosinophilic/exacerbator phenotypes, and never use it as monotherapy in COPD.



## 9. DAILY REHAB — pulmonary rehab

### WHAT YOU SEE

The 'DAILY REHAB' sign up top.

### WHAT IT ENCODES

Pulmonary rehabilitation (supervised exercise + education + breathing techniques) improves dyspnea, exercise capacity, and quality of life and reduces hospital readmissions, especially when started after an exacerbation. It does not change FEV1 but improves function.

## 10. Crossed sign — smoking cessation

### WHAT YOU SEE

The red crossed-out sign on the boards.

### WHAT IT ENCODES

Smoking cessation is the SINGLE most effective intervention in COPD — it slows the accelerated rate of FEV1 decline back toward normal. Along with long-term oxygen (in qualifying hypoxemia), it is one of the only interventions proven to reduce MORTALITY.

### BOARD TRAP

Inhalers relieve symptoms and reduce exacerbations but do NOT change the disease course or mortality the way quitting smoking does. Only smoking cessation and LTOT (in hypoxemia) reduce mortality.

## 11. Wheelchair player — LTOT & cor pulmonale

### WHAT YOU SEE

The player in the wheelchair.

### WHAT IT ENCODES

Advanced disease → chronic hypoxemia → pulmonary HTN → cor pulmonale (right heart failure). Long-term oxygen therapy ( $\geq 15$  h/day) reduces MORTALITY when resting  $\text{PaO}_2 \leq 55$  mmHg or  $\text{SpO}_2 \leq 88\%$ , OR  $\text{PaO}_2 \leq 59$  /  $\text{SpO}_2 \leq 89\%$  WITH cor pulmonale, RHF, or erythrocytosis (Hct  $> 55\%$ ).

### BOARD TRAP

LTOT mortality benefit applies only to RESTING hypoxemia meeting criteria — exercise-only or sleep-only desaturation does NOT qualify for the survival benefit.

## 12. Green germs — exacerbation triggers

### WHAT YOU SEE

Green bugs scattered on the ice.

### WHAT IT ENCODES

Exacerbations are most often triggered by respiratory infections — viruses (rhinovirus, influenza) and bacteria (Haemophilus influenzae, Streptococcus pneumoniae, Moraxella catarrhalis); air pollution is another trigger. Prevention includes influenza, pneumococcal, COVID, and pertussis vaccination.

## 13. The brawl — acute exacerbation

### WHAT YOU SEE

The crowd fight breaking out at the top.

### WHAT IT ENCODES

Acute exacerbation = increased dyspnea, increased sputum VOLUME, and/or increased sputum PURULENCE (the 3 cardinal Anthonisen criteria). Treat with: bronchodilators (SABA +/- SAMA), systemic steroids (prednisone ~40 mg x5 days), and antibiotics when  $\geq 2$  cardinal features (including purulence) or mechanical ventilation. Consider NIPPV if respiratory acidosis (pH  $< 7.35$ ).

### BOARD TRAP

A SHORT oral steroid course (~5 days) is sufficient — no prolonged taper for an exacerbation. Antibiotics are added based on cardinal features/purulence or ventilatory support, not for every exacerbation.